

WHAT INPURSUIT IS

InPursuit is the *infrastructure*
for person–centric clinical services.

Not a platform. Not a SaaS. Not another wellness app.

\$3.5M, once. App live weeks from close. Both engines by Q4.

First Wave

SELECTED BY CMS
ALONGSIDE AMAZON, APPLE & GOOGLE • WHITE HOUSE
• APRIL 2026

22,000

PATIENT ANCHOR • SIGNED
HONEYCOMB MEDICAL GROUP • BEFORE THE ROUND
CLOSES

\$300K

FILED ON A \$50B FEDERAL FUND
RURAL HEALTH TRANSFORMATION • DECISION
PROJECTED AUGUST

InPursuit Health

BUILT • SIGNED • FILED –
THE WIRE IS THE LAST DEPENDENCY

HEALTH & WELLNESS APPS HAVE FAILED • FEDERAL POLICY JUST CHANGED

Both sides waited for the same fix. *The window just opened.*

IF YOU'RE A CONSUMER

\$1,092_{/yr}

Spent on 3–7 disconnected apps — none talking to each other.

IF YOU'RE A PRACTICE

76%_{gone}

Of portal downloads opened once and abandoned.

THE SUPPRESSED SIGNAL

83.1%

Concierge & DPC growth in five years — the segment built to escape the system.

01 • Federal Policy

Genie's out.

CMS returned data ownership to patients — structural, not reversible.

02 • Consumers

The shift.

73% want control of their data. Only 34% want the next visit in an office.

03 • Practices

The BOOM.

2026 PFS codes flow to independent NPIs. 230K practices are ready.

04 • Payers

They're paying.

APCM, CCM, RPM, TCM concurrently billable — MA parity, commercial follow.

Four tailwinds. One infrastructure layer. *One window — and it is open now.*

WHAT INPURSUIT IS · HOW WE DELIVER IT

InPursuit is not just an app. *It is the infrastructure for personalized health & wellness — supporting both consumers and providers.*

WELLNESS

Save Money · Personalized Insight
Lifestyle · Wearables · Biometrics
AI · Clinical Services



InPursuit Health

HEALTHCARE

New Digital Clinical Services
Better Outcomes

ENGINE 01 · D2C APP MEMBERSHIP

The InPursuit App

A consumer membership that gives every American ownership of their health data — and pays them back for using it.

HARNESS THE POWER OF YOUR DATA™

ENGINE 02 · THE PARTNER NETWORK

The InPursuit Partner Network

The infrastructure that lets independent practices offer digital clinical services they couldn't deliver on their own — and get paid for them.

RESULTS MATTER™

THE CONSUMER SIDE

330_M

Americans — finally able to own and use their own health data

80% favor **remote monitoring** of their health by their care team.

NATIONAL SURVEYS, 2022-2024

THE PROVIDER SIDE

230_K

Small & mid-size practices — finally able to offer digital clinical services

65% of patients would **switch doctors** for a better digital experience — up from 55% the year before.

TEBRA PATIENT PERSPECTIVES REPORT, 2025

CONCIERGE LAYER

Serving the fastest-growing segment in American healthcare — the people and clinicians who pay for time, access, and continuity the system can't deliver. One stack, three audiences, one company.

ENGINE 01 · D2C APP MEMBERSHIP

Harness the Power of *Your Data.*[™]

A universal, all-payer app on Apple App Store, Google Play, and the Medicare App Library — one membership that provides *personalized care and wellness*, regardless of what you are InPursuit of.

01 · PERSONALIZED

Care built for *you*.

Your record, your data, your wellness goals — **guidance tailored to your life.**

02 · SAVE MONEY

Spend less, *get more*.

Hidden savings on Rx, procedures, and benefits you didn't know you had — **\$5K+ avg/yr.**

03 · SAVE TIME

Hours back, *not lost*.

No more portal logins, hold music, or paper forms — **42+ hrs recovered/yr.**

04 · YOUR FEED

A living health *feed*.

New insights, content, and offers added **continuously** — not a one-time download.

ONE MEMBERSHIP LADDER · INTELLIGENCE GATES CLINICAL

FREE

\$0

Your record, in one place

PLUS

\$14.99

Every device, talking

FAMILY

\$34.99

Up to 4 · one app

TOTAL INTELLIGENCE

\$49.99

The individual flagship

TOTAL FAMILY INTELLIGENCE

\$89.99

Household flagship · up to 4

+ CONCIERGE

\$78.99

Full stack · family \$139.98/household

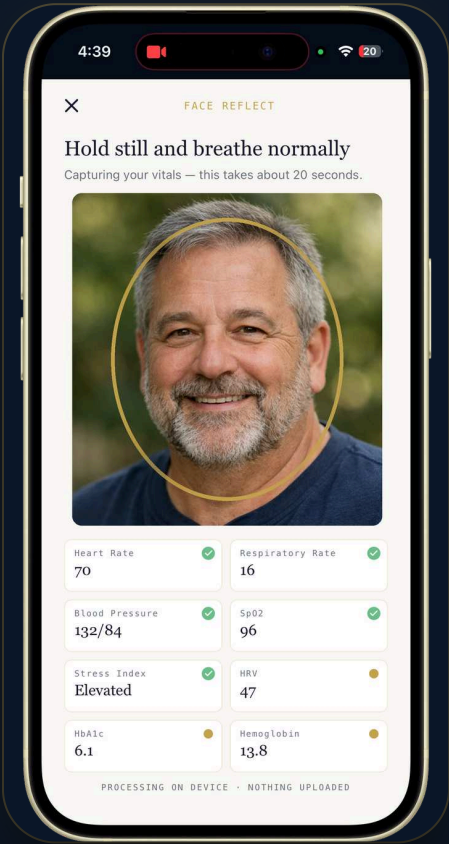
\$50/mo in → \$400–\$700/mo back

In Rx savings, benefit recovery, brand cashback, and personalized offers.

INSIDE ENGINE 01 · THE D2C CONVERSION ENGINE

InPursuit Reflect™

How InPursuit converts D2C members from Free to Plus — and earns the right to deliver clinical services.



FACE REFLECT™ · 50-SEC SCAN
ON-DEVICE · NOTHING UPLOADED

21 Metrics. One Scan. Clinical-Grade Accuracy.

7 FDA-CLEARED · 4 CLINICALLY VALIDATED · 10 WELLNESS & RISK SIGNALS



Outperformed Apple Watch ECG & leading 6-lead accessories in 2025 head-to-head — 5.6% insufficient reading rate vs. 13.5% Apple Watch.

D2C ENGAGEMENT ENGINE · REFLECT PRICING

Free

\$0 / mo
10 scans / mo

Try it. Keep your record.

Plus

\$14.99 / mo
Up to 300 / mo

+ wearable sync · trends · alerts

UNLOCK THE HUB

Every Wearable. Every App. *One Place.*

REFLECT IS THE BIOMETRIC HUB — YOUR SCANS + EVERY DEVICE YOU ALREADY OWN.

21+
INTEGRATIONS
AT LAUNCH

- Apple HealthApple WatchOura RingGarminWhoopFitbit
- Samsung HealthGoogle FitWithingsDexcomPolar... and more

Every scan deepens the record. Free converts to Plus — and daily scans are the data Engine 2 clinicians need to bill CCM, RPM, and BHI.

INSIDE ENGINE 01 · CASH-PAY CLINICAL · VS RO & HIMS

Ro and Hims charge \$149 before the medication. *Our whole stack is \$158 — all-in.*

Same category — and a clinician who actually has the member’s data. Ro and Hims bill \$149/mo for the telehealth wrapper alone; medication is separate, so real cost runs \$300–\$600. Our \$79 clinical program is an add-on billed on top of the \$78.99 Total Intelligence + Concierge base — \$158 all-in, intelligence included.

RO · HIMS — STANDALONE

\$149

/mo

membership alone — medication billed separately

+ \$149–\$399/mo medication

≈ **\$300–\$600/mo all-in**

Async text · single condition

No unified record · no continuity

RO & HIMS PUBLISHED PRICING · JUL 2026

INPURSUIT — ON THE INTELLIGENCE LAYER

\$158

/mo all-in

\$78.99 TI + Concierge base + \$79 clinical program on top

The program is an add-on — **never sold standalone**

Concierge clinician from your complete record

Every device, every lab · unified continuity

56% contribution margin

\$158 VS \$300–\$600 · THE WHOLE INTELLIGENCE LAYER

ONE MODEL · THREE PROGRAMS

Each billed on top of Total Intelligence + Concierge (\$78.99).

GLP-1 / WEIGHT

all-in \$158

\$79

56%

MEN’S HEALTH

all-in \$158

\$79

51%

WOMEN’S HEALTH

all-in \$138

\$59

56%

The clinical program the member already wanted — captured on infrastructure that’s already paid for.

~\$1.61M cash-pay line

~1,765 members · ~1% of base

~55% blended contribution

Intelligence gates clinical — the layer Ro and Hims can’t build.

CMS HEALTH IT ECOSYSTEM · FIRST WAVE PARTICIPANT · APRIL 9, 2026

Three channels. *67million Medicare lives.* A front door most pitches don’t have.

First-Wave selection into the CMS Health IT Ecosystem opens a direct distribution path to every Medicare beneficiary in America — the highest-value cohort in D2C digital health, reached through a CMS-promoted channel — not cold paid media alone.

APRIL 9, 2026 · WHITE HOUSE LAUNCH

CMS Health IT Ecosystem · *First Wave Participant*

Selected alongside Amazon, Apple, Google, Oracle Health, Epic, and 55+ other organizations. Pledged categories: **Conversational AI, Kill the Clipboard, Chronic Care.**

THREE DISTRIBUTION CHANNELS · ONE APP

	Medicare App Library CMS-promoted directory · InPursuit lists at app launch	67M BENEFICIARIES
	Apple App Store Universal consumer reach · iOS	D2C ALL PAYERS
	Google Play Universal consumer reach · Android	D2C ALL PAYERS

WHY MEDICARE IS THE RIGHT COHORT

- **Highest chronic disease prevalence** — 95% have 1+ chronic condition, 85% have multiple
- **Already conditioned** to digital health tools and remote monitoring
- **Highest device ownership** in the population — the Reflect hub thesis at scale
- **Concierge converts higher here** — clinical access is the actual unmet need

THE ADVANTAGE

a CMS-promoted channel most D2C pitches don’t have

CONSERVATIVE 12-MONTH PROJECTION

0.25% penetration into the Medicare App Library cohort — *the floor most digital-health pitches use.*

168,500

MEMBERS BY
MID-2027

D2C ARR EXIT RUN-RATE

168,500

\$3.67M

+ ENGINE 2 CONVERSION UPLIFT

\$257K ARR

503 Concierge members × \$42.50/mo blended Engine 2 conversion (CCM, RPM) — billed through IPMC, independent of the membership fee.

COMBINED EXIT ARR · MID-2027

\$3.93M

\$3.67M D2C · \$257K ENGINE 2 UPLIFT

Most pitches project 0.25% penetration. We have the distribution channel that most pitches don’t.

A *membership* for the practices that built American medicine.

WHAT MEMBERSHIP DELIVERS

Membership in the InPursuit Partner Network provides the 230,000 small & mid-size practices with the ability to finally deliver digital care, modern technology, and multi-payer revenue — *without building it alone.*

THREE LAYERS • FIFTEEN SERVICE LINES • FUNDED BY CARE, NOT FEES

01
The *operating* layer

Eligibility and enrollment, documentation and billing, and a **care team staffed for the practice.** There is no one to hire, nothing to buy, no implementation to survive.

STAFFED • ALREADY RUNNING

02
The *patient* layer

The app, the unified record, and between-visit engagement, **live across the whole panel** — with FDA-cleared vitals captured from the phone the patient already owns.

PANEL-WIDE • EVERY PAYER

03
The *care engine*

Fifteen service lines — cardiometabolic, respiratory, renal, behavioral, transitions — turn on one, add the rest with **no new contract and no new integration.**

ONE CONTRACT • ONE INTEGRATION

04
Delivery, *their way*

Full Handoff: InPursuit delivers and bills every digital line. **Split:** already run a line? Keep it, bill it, keep 100% — InPursuit delivers the rest. Chosen line by line.

TWO MODELS • ONE MEMBERSHIP

05
Independence, preserved

Practice keeps the patient relationship. Practice keeps 100% of existing FFS. **Not an Aledade. Not a health system buyout.** A membership.

ADDITIVE • NOT EXTRACTIVE

InPursuit Growth • InPursuit-funded | *InPursuit Signature* • \$1/patient/yr • practice-branded

FUNDED BY CARE, NOT FEES

THE INPURSUIT NETWORK • HOW CARE GETS DELIVERED

Practice refers. *IPMC delivers and bills.*

Every IPN member runs on the same model: the practice refers patients into the digital services it can't deliver alone, and InPursuit Medical PC (IPMC) takes care of everything else — delivery, billing, and staffing — under its own NPI. No fee for care delivery ever flows from the practice to InPursuit.

EVERY PATIENT GETS The *same* coordinated care — chronic care, monitoring, outcomes. SAME MODEL, EVERY MEMBER

01

THE PRACTICE

Refers the patient.

The practice identifies eligible patients and refers them into the InPursuit Partner Network. **It keeps the patient relationship** — and sheds delivery, staffing, and billing entirely.



02

INPURSUIT MEDICAL PC

Delivers the care.

IPMC's NP-led clinical team delivers **CCM, RPM, APCM, BHI, TCM** — with FDA-cleared vitals capture, enrollment specialists, and a complete RCM back office. The infrastructure the practice couldn't build alone.



03

THE PAYER

Pays IPMC directly.

IPMC bills Medicare, MA, commercial, Medicaid MCOs, and cash — **under its own NPI**. Each claim is for services IPMC actually performed; the practice is never a party to the billing.



04

INPURSUIT → YOUR CLINICIANS

Pays *FMV* for supervision.

Where your clinicians supervise InPursuit's clinical team, InPursuit pays the practice a **flat, fair-market-value supervision fee** — the only recurring payment in the model. It keeps your clinician on the case, and it's why patients say yes. Never a percentage of anything.

PRACTICE KEEPS THE PANEL

IPMC OWNS DELIVERY

IPMC'S NPI • MULTI-PAYER

THE ONLY RECURRING PAYMENT

ANTI-KICKBACK • BY DESIGN

The practice never pays InPursuit for care delivery — no fee, no percentage of collections or savings. IPMC bills its lines under its own NPI; the practice bills its own work and keeps 100%. The only recurring payment is a flat fair-market-value supervision fee to partner clinicians for oversight actually performed — structured to each state's CPOM and the AKS personal-services safe harbor, 42 CFR §1001.952(d). Every partner in this market sends the practice an invoice; we send the practice a check.

ENGINE 02 • PROOF OF REAL CUSTOMER

The care Honeycomb's 22,000 patients finally get.
230,000 practices need the same.



ANCHOR PRACTICE
FOUNDING MEMBER • IPN

Honeycomb Medical Group

MEMPHIS, TENNESSEE • ECLINICALWORKS EMR

22,000

FULL PATIENT PANEL • MULTI-PAYER

Funds ~\$2.08M/yr of care by Year 3

Anchor-scale — a 22,000-patient practice, not a typical member. Conservative floor. Commitment signed; onboarding begins within 4 weeks of close — the proof Engine 2 is real before a dollar of the round is spent.

THE PANEL

~7,700 RPM-eligible • ~5,500 CCM-eligible • ~3,300 BHI-eligible

Across every payer type. **Patients Honeycomb structurally cannot serve with clinical-services programs alone.** InPursuit delivers infrastructure; Honeycomb refers and keeps the panel.

THE MODEL

No SaaS fee. No per-patient fee. *No percentage of revenue.*

Honeycomb never pays a fee for care delivery — never a percentage of collections or savings. InPursuit Medical PC delivers CCM, RPM, APCM, BHI, TCM and bills payers directly under its own NPI — across Medicare, MA, commercial, Medicaid MCOs, and cash. Honeycomb refers patients, keeps the relationship, and sheds delivery, staffing, and billing entirely.

Honeycomb is the proof point — not the ceiling.

GRANT-FUNDED ACQUISITION · NON-DILUTIVE

\$50 billion is moving to the states. *We’ve already filed.*

The Rural Health Transformation Program funds a rural practice’s entire InPursuit infrastructure — the state pays, not the practice. Every first-year dollar must be obligated by October 30, 2026. That deadline is pushing money to practices now.

\$50B

OVER FIVE YEARS

~\$200M

AVG FIRST-YEAR AWARD

50 states

LIVE NOW

Oct 30, 2026

OBLIGATION DEADLINE

ALREADY FILED · TIER 1 · MISSISSIPPI · \$205.9M FIRST-YEAR AWARD

\$300K submitted for Honeycomb’s Mississippi footprint

Honeycomb as directly eligible applicant · InPursuit as named technology partner · awards projected August 2026

The playbook, proven once — ready to repeat across 50 states.

1• The state pays for the infrastructure

2• The practice gains the capability · InPursuit is paid at onboarding

3• The template repeats, state by state

YEAR 3 • FULL HANDOFF BASIS • COMBINED (ENGINE 1 + ENGINE 2)

Two engines. *One per-member basis.* One cap table.

YEAR 3 • ANCHOR • 25 MEMBERS

~\$35M *combined y3*

25 IPN members, each modeled at ~10K-equivalent panel · Engine 1 held flat at Y1 exit ARR · Full Handoff basis · CIN and D2C scaling held as upside.

E2	~\$31.2M	25 members × ~\$1.25M per ~10K-panel member · IPMC bills under its own NPI · Honeycomb carried at conservative ~10K-equivalent
E1	~\$3.67M	D2C membership held flat at Y1 exit ARR · 167,500 base members + 1,000 Total Family Intelligence households (TFI additive)
E2+E1	~\$35M	Combined Year-3 run-rate · conservative basis · per 260806.2 reconciled across exec summary, IPN, and D2C source docs

THE Y3 LADDER • THIS UNIT STACKS

Each rung = (members × ~\$1.25M Engine 2) + ~\$3.67M Engine 1 flat. Conservative basis throughout.

5 MEMBERS	~\$9.9M
15 MEMBERS	~\$22M
25 MEMBERS • ANCHOR	~\$35M
50 MEMBERS	~\$66M
100 MEMBERS	~\$128M

UPSIDE EXCLUDED: CIN, HONEYCOMB TRUE 22K, D2C SCALING

SOURCE: COMBINED REVENUE PROJECTIONS 260806.2 • FULL HANDOFF BASIS • CMS 2026 PFS FINAL RULE • D2C MEMBERSHIP STRATEGY V2.1 • FULL MODEL IN INVESTOR PORTAL

WHY NOW · WHAT EXECUTION LOOKS LIKE

What your wire sets *in motion*.

A single close starts every clock at once. Nothing below is aspiration — the app is built, Honeycomb is signed, and the Mississippi application is filed. The wire is the last dependency.



DAY 0

Single close.

\$3.5M in · Class C issued · every
workstream funded on day one.



WEEKS 4-6

The app goes live.

App Store, Google Play, Medicare App
Library — first D2C collections.



AUGUST 2026

Mississippi decides.

\$300K filed with Honeycomb — award
decision projected this month.



~DAY 60

Honeycomb enrolling.

22,000-patient panel · Engine 2 revenue
under IPMC's own NPI.



Q4 2026

Both engines live.

Consumer + Partner Network producing —
next round optional, not required.

WHAT THE FIRST 180 DAYS PROVE

Real conversion — App Library penetration and
Free → Plus → Concierge rates, measured, not modeled.

Real enrollment — Honeycomb's panel converting under the
FMV supervision model, live.

The second practice — proof the anchor is a template, not a
one-off relationship.

The three questions every diligence asks — answered with data before the next round is even a conversation.

FOUNDERS & LEADERSHIP

The team that is *InPursuit of building* a Person-Centric Healthcare Ecosystem.

FOUNDERS



Bennett McPhatter

USMC VETERAN

Co-Founder & CEO

Built and sold Visual Analytics to Raytheon. 30+ years of data platforms across DoD, intelligence, and the Fortune 500.



Chris Ingram

Co-Founder & CPO

Built InPursuit's neural-network predictive engine. Former Vice President, MSCI.



Hongda Lin

Co-Founder & CTO

25+ years architecting federated search and data orchestration for U.S. intelligence and federal healthcare.



Patrick Truxillo

Co-Founder & CSO

Healthcare interoperability strategist. 20 years connecting disparate data across DoD health programs and government.

EXECUTIVE TEAM



Dr. Joseph M. Chalil

U.S. NAVY VETERAN

Chief Medical Officer

20+ years health policy. Three U.S. patents. IAOTP Top Physician of the Year.



Jose Arrieta

Chief Innovation Officer

Former U.S. Federal Chief Data Officer. Blockchain supply chain pioneer. National data modernization leader.



Michael Mark

AIR FORCE VETERAN

CRO — VBC

Former healthcare provider and payer CEO with decades building healthcare partnerships. Leads InPursuit Partner Network go-to-market strategy.



Alison Cossette

Chief AI Officer

Responsible AI expert. Graph data science, provenance systems, trustworthy AI architecture.

CONVICTION CAPITAL

Executive team is currently engaged on a **fractional, uncompensated basis** — conviction capital. Founders are compensated at close; the broader executive team remains **fractional** until revenue supports full comp.

Also: Andy Hathorn (CFO) · Joseph Higbee (Chief Marketing Officer) · Jeff Sirianni (COO) · David Janet (Legal Advisor)

Veteran-owned | American-built | Every line of code written in the USA

THE INVESTMENT THESIS · WHAT \$3.5M ACTUALLY BUYS

\$3.5M seed. App live in weeks. *Both engines by Q4.*

To hit the Year-3 anchor of ~\$35M combined, *InPursuit needs* twenty-four more members like Honeycomb. *Not 24,000. Not 2,400. Twenty-four.* The technology is built, the compliance posture is in place, and Honeycomb is signed. What \$3.5M funds is not technology development — it's the launch that turns built infrastructure into recurring revenue across both engines.

NOW · THE SEED ROUND

\$3.5M

Class C Units NEW UNITS · PRICED EQUITY	\$20M Pre-Money \$23.5M POST-MONEY
STRUCTURE Priced equity	MINIMUM CHECK \$25K
CLOSE Single close	CONVERSION C-Corp · QSBS

Class C units convert at the C-Corp conversion, where QSBS eligibility begins. Next round optional, not required — operations increasingly carried by collections.

USE OF PROCEEDS

Everything starts on day one. *Nothing waits on anything else.*

~\$100K	The app to the stores · live 4–6 weeks after the first \$100K clears
~\$900K	Clinical services stand-up · staffed ahead of collections
~\$350K	Partner delivery capacity · Engine 2 on · Honeycomb live
~\$900K	Team & production · key hires, cloud & AI, payer integrations
~\$300K	Member acquisition & marketing · ignites at launch
~\$400K	Corporate foundation · legal, insurance, compliance, finance ops
~\$550K	Operating reserve & working capital · bridging the collections lag

DOCUSIGN · SINGLE CLOSE · \$3.5M TOTAL

INVEST@INPURSUITHEALTH.COM

"What \$3.5M actually buys: ownership in the infrastructure for a person-centric healthcare world. And the capital to scale from one Honeycomb to 25 practices on infrastructure designed for unlimited scale."

INVEST@INPURSUITHEALTH.COM
HARNESS THE POWER OF YOUR DATA™ · RESULTS MATTER™